



3.1 Resident Rights.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens shall conduct its business with the primary concern for preserving and recognizing the dignity and individual rights of our residents. The resident's rights shall not be limited in any way by the Community or its team members, except where it may be necessary for the health and safety of the resident, other Community residents, team members and visitors. In such cases, the resident will be fully advised of applicable rules or policies.

Definitions

Bill of Rights

A list of resident rights for Merrill Gardens assisted living residents. Generally included with the Residency Agreement.

Guidelines

Community Commitment

The Community abides by the residents' rights as outlined in Exhibit VII of the Residency Agreement, (Resident Rights) and by state or federal statute as appropriate for the Community's Licensure status and location.

GA: The General Manager will be responsible for ensuring that the rights of all residents are scrupulously safeguarded in accordance with the Rules and Regulations for Assisted Living Communities other applicable Federal and State laws and in particular reporting as dictated in O.C.G A 31-8-50. Article 4.

Effort shall be made to preserve the privacy of the resident. All communications and records pertaining to resident care shall be treated as confidential. Access to such records shall be granted only to government agencies possessing statutory authority, healthcare providers for the purpose of coordination of care, and to individuals or other parties upon receipt of the resident's written permission or upon written order of a court of an appropriate jurisdiction.

Communicating Resident Rights

Every assisted living resident is provided a list of Resident Rights as part of the Residency Agreement, Exhibit VII, and acknowledges such in the addendum Receipt of Referenced Documents. Communities will adopt it verbatim. In the event that Exhibit VII, Resident Rights fails to address state regulation, contact the Vice President of Operations. The Resident Rights format must be consistent and must offer state agencies and Merrill Gardens alternatives for grievances if problems arise.

Additionally, the Community will take the following actions:

1. Assisted living residents will acknowledge their receipt of a copy of Resident Rights upon moving into the Community.
2. Team members will review and receive instruction regarding Resident Rights.
3. This training must be documented in the team members' files.
4. A framed copy of Resident Rights will be posted near the front desk.
5. Other methods of communication will be developed and implemented as necessary for residents, visitors, team members and the general public that state the Community's position regarding resident rights and their protection.

GA:

- A. *As a part of the admission process, the General Manager will discuss residents' rights in detail with the prospective resident and/or prospective resident's representative or legal surrogate. This discussion will include the fact that the rights cannot be waived by the resident or the resident's representative or legal surrogate, if any. The discussion will also include the facility's responsibilities under Georgia law concerning the effectuation of a "Do Not Resuscitate" order, and the fact that under no circumstances will physical or chemical restraints (including bed rails which cannot be raised and lowered by the resident without assistance, and chair restraint devices which cannot be released by the resident without assistance), isolation, or confinement for behavioral control be used on any resident at any time. The resident and/or the resident's representative or legal surrogate will receive a copy of the Residents' Rights document and will acknowledge receipt by signing and dating the last page of the document, a copy of which will be maintained in the resident's individual file.*
- B. *All staff members will receive training in residents' rights and the Long-term Care Abuse Reporting Act within the first week of employment. Each employee shall be provided a copy of both Residents' Rights and Rules and Regulations for Assisted Living Communities Care Homes) as detailed CHAPTER 111-8-63.25 RULES AND REGULATIONS FOR ASSISTED LIVING and have access to them in this policy manual, and review Article 4, Reporting Abuse or Exploitation of Residents in Long-term Care Facilities (O.C.G.A. §31-8-50 et. seq.), and acknowledge receipt by signing and dating the last page of the documents. A copy of each signed document will be maintained in the employee's personnel file.*
- C. *Staff will receive ongoing training through staff in-services specifically addressing residents' rights and employee responsibilities under the Long-term Care Abuse Reporting Act in addition to initial training at orientation.*
- D. *The General Manager will be responsible for ensuring that all employees, not just those employees who offer direct care to the residents, maintain awareness of each resident's rights, health and safety; and intervene if a resident's rights, health or safety are at issue, or if there are any violations that need to be reported and addressed.*
- E. *The General Manager will strive to ensure that all volunteers who have direct resident contact of a regular basis have a criminal background check, TB testing, and have been oriented appropriately to help ensure the resident rights, safety and reporting of injury, emergency procedures, universal precautions, confidentiality and policy on no assistance with personal care and other topics required by the state and community.*

- F. The General Manager will coordinate closely and regularly with the Long-Term Care Ombudsman to take advantage of the Ombudsman's expertise in the area of residents' rights and will seek the Ombudsman's assistance in conducting the quarterly continuing education training in the area of residents' rights. All employees are encouraged to converse candidly with the Ombudsman whenever the facility is visited, or telephonically as needed. The Ombudsman contact poster will be displayed conspicuously on the common area bulletin board.*

Complaints and Community Actions

The Community will provide avenues through which the resident may file confidential complaints. The resident will also be provided a list of agencies and advocacy groups outside the Community with whom they may file complaints.

If a resident violates the rules or policies of the Community, as outlined in the Residency Agreement and in the Resident Handbook, actions taken regarding the violation(s) will be handled tactfully and respectfully. At no time will the resident be disciplined. Residents will be treated as adults who have a contractual relationship with the Community. Actions that cause a breach of the terms of the contract will be handled appropriately, starting first with an explanation of the violation. Persistent failure to adhere to rules or policies may result in warning letters, problem solving meetings, and possible eviction in accordance with applicable state regulations. In such cases, involvement of the Vice President of Operations is mandatory.

GA: Residents are encouraged to form a Resident Council to monitor the maintenance of residents' rights and to provide input to the General Manager regarding problems, facility activities, schedules, and resident interests. It will be the responsibility of the General Manager to help ensure that meetings of the Resident Council may be held outside the presence of owners, management, or staff members of the community, and to assist the Council to the extent practicable in the conduct of its meetings.

What to Do If You Suspect a Resident's Rights Have Been Violated

If you suspect a resident's rights are being violated or they are suffering from abuse at the hands of a team member, the resident's family, or others:

- Report the suspected abuse immediately to the General Manager or their designee, and as required by law.
- An official report must be filed within a stated time frame as required by state regulations. Refer to Suspected Abuse of Resident and Team Members.
- The community team members must attempt to protect residents from repeat abuse incidents, which may include efforts to prevent contact with a resident suspected of abuse and/or evicting that resident from the community.

3.2 Release of Resident Information and Confidentiality

.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens seeks to maintain the privacy of all its residents. This includes privacy regarding release of information to the media or general public, and confidentiality—both within and outside the Community—regarding a resident's personal or medical information. Resident information is treated as confidential, and information will not be released without written permission from the resident or legal representative.

Scope

Release of Information

The resident must provide written approval, using the Confidentiality and Disclosure of Resident Information form, or equivalent, to release information regarding the resident's personal and/or medical information to:

- General public inquiries
- Media inquiries
- Media releases related to specials events at Merrill Gardens
- Family members or others who do not have legal guardianship, Power of Attorney or conservatorship

The Confidentiality and Disclosure of Resident Information form also provides for release of information from the resident's home health providers, private duty aides or companions, if applicable. This allows for the Community to have access and authorization for medical information regarding a resident.

Confidentiality

Merrill Gardens documentation procedures and policies are designed to protect the confidentiality of information for each resident. Resident information may be released to legal entities authorized to have such access. This includes regulatory agencies, legal guardian, conservator, Power of Attorney, spouse, and requests through subpoena. Additionally, the Community is authorized to discuss resident's health issues with the attending physician or medical emergency team, and other care providers.

In the event of a resident's death, entry into their apartment must have been previously authorized in writing in Exhibit V of the Residency Agreement, Confidentiality and Disclosure of Resident Information form, or by one or more of the above legal entities. The General Manager or their designee is responsible for ensuring proper access to the apartment of a deceased resident by an identified party.

3.3 Resident Transfers and Lifting

.....Revised 10/24/2018 Reviewed 10/24/2018

Policy Purpose

Merrill Gardens supports the independence of our residents, while recognizing that some residents may need assistance with transferring from one location/surface to another, such as from a chair to standing. Based on an evaluation of the resident's needs and appropriateness, we provide assistance with routine transfers. Our team members will not lift a resident who has apparently fallen and appears to be injured, because of the risk of injury to both the resident and team member(s) involved.

Definitions

Adaptive Equipment

Mobility aids which the resident owns or rents, such as walkers, canes, wheelchairs, transfer poles, etc. Team members should be trained in their use before assisting the resident with them. Team members will not assist with mechanical lifts.

Assist

Helping a person move from one location to another through either verbal or physical cuing. (i.e. physical cueing through a light touch on the back or hand). Verbally directing a resident where to place feet and hands. Assistance may also be in the form of placing adaptive equipment or stabilizing items in front of or next to a person.

Lift

One trained caregiver moving a person from one location to another by offering minimal physical assistance. The resident must be able to bear weight and participate. (i.e., physically support movement from sitting to standing).

Stabilizing Items

Sturdy items the resident uses to assist with mobility such as a chair.

Transfer

Moving a person from one location or surface to another. (i.e., moving from a bed to a chair. Moving from a seated position to standing).

Guidelines

Resident Evaluation for Ability to Transfer

It is preferable for residents to transfer independently with or without adaptive equipment, as appropriate. Residents needing assistance for transfers are evaluated for risk of falls.

Types of Assistance Allowed

Merrill Gardens will provide assistance with routine transfers. The ability of the community to provide this service will depend on an evaluation of the resident and the availability of trained team members. Routine transfers include those transfers needed in the usual course of completing activities of daily living. This would include: getting in and out of bed, toileting, getting in and out of a chair, and getting in and out of the shower. It does not include assistance with rising after a fall or similar emergent events when the resident appears to be injured or has pain, because of the risk of injury to both the resident and team member involved. Each function can be defined by the level accepted. A current resident who has a short-term need, or who has developed a need for two-person assist, may be retained if permitted by state regulation and when the community can meet their needs.

MINIMAL ASSIST:

Verbal and/or physical cueing. The resident is able to initiate and complete the transfer with verbal instruction and/or touch to prompt. The team member does not support the resident's weight or physically move any part of the resident.

MODERATE ASSIST:

May include verbal and/or physical cueing and transferring. One team member may occasionally physically assist the resident to move. The resident must be able to support their own weight and participate in the transfer. The team member assisting with the transfer must have been trained in resident transfers.

EXTENSIVE ASSIST:

May include verbal and/or physical cueing and transferring. One team member may frequently physically assist the resident to move. The resident must be able to support their own weight and participate in the transfer. The team member assisting with the transfer must have been trained in resident transfers.

TOTAL ASSIST:

This type of transfer is not to be provided by Merrill Gardens' team members. The resident cannot physically support their own weight or initiate the movement. If adequate private duty care provider support is not available, the resident may not be appropriate for residence at the community. An exception may be made for residents on hospice, with GM approval, and where permitted by regulation.

Cueing

VERBAL CUEING:

A caregiver is present and gives the resident instructions to complete the transfer without any physical contact. This could include reminders to make sure their feet are flat on the floor, that they are wearing non-skid footwear, that they sit at the edge of the bed or other surface for a moment before standing, and/or that they need to use an adaptive device.

PHYSICAL CUEING:

A caregiver is present and physically participates in the transfer without lifting or actually moving the resident. This may include placing a chair, providing adaptive equipment, and/or lightly touching the part of the body the resident needs to move. The caregiver does not lift or support any of the resident's weight.

Transferring

General Transferring Guidelines:

- The caregiver(s) shall receive training in proper transferring techniques.
- If a gait belt is used, apply it snugly around the resident's waist, but loose enough for 2 fingers to fit under it.

AL: Gait belts may not be used.

- At any time, if the caregiver feels they or the resident are not able to complete the transfer, they should discontinue, help ensure the resident is in a safe position and call for assistance. If another caregiver is not able to assist with completing the transfer, or the resident is in pain or appears to be injured, the nurse should be called to evaluate the resident before moving them. If a nurse is not in the Community, call 911 for evaluation.
- Routine transfer assistance must be included in the Evaluation and indicated on the Service Plan.

Assisting a Resident from a Seated Position:

- The resident should be in a seated position with feet slightly apart and on the floor. Footwear should be non-skid. Adaptive devices such as a walker should be placed within reach.
- Stand closely in front of the resident with feet slightly apart and knees on each side of the resident's knees for support.
- Bend at the knees, keeping the back as straight as possible.
- Grasp the resident around their back (like a hug).
- Ask the resident to rock forward a specific number of times (usually once or twice) to gain momentum for standing. Work with the momentum and cue the resident to stand, while providing support with the knees.
- Pause long enough for the resident to get their balance while standing.
- If they are going to move to another location/surface, walk with them while keeping at least one hand under their forearm/elbow area for support.
- Sitting is the reverse of standing. Help ensure the back of the resident's legs are touching the surface they are going to sit on. Grasp the resident around the waist, as above, with both hands. Brace the resident's knees as above. Cue the resident to place their hands on a rail, armrests or solid object, if available. Cue the resident to sit and guide the resident to a seated position. Keep knees bent and back as straight as possible while assisting the resident to sit.

Assisting a Resident with Rising after a Fall:

- Evaluate the scene. Ask the resident if they have any pain. If they indicate they have pain or appear to be injured, do not move them until they are evaluated by a qualified health care professional. If a nurse is in the community, call them to evaluate the resident. If a nurse is not in the community, call 911.

- If the resident does not appear to be injured or in pain, they may be assisted to get up off the floor.
- Verbally cue the resident through each step, allowing them to pause after each move to regain balance and strength.
- Assist the resident to roll to their side.
- Have the resident bend the knee of the leg that is on top, so that the knee and foot rest on the floor. If the floor is too hard, a pillow or folded towel may be placed under their knees.
- Place a sturdy chair or stool in front of the resident where they can reach it to use to help pull themselves up.
- Have the resident use their arms to roll forward and push up to a kneeling position.
- Once in a kneeling position, the resident should bend whichever knee is stronger and place that foot on the floor, keeping the other knee on the floor.
- Assist the resident to rise slowly and turn to sit on the chair or stool.
- Re-check the resident for any pain or other evidence of possible injury
- Complete an incident report.

3.4 Infection Control, Standard Precautions and Communicable Diseases

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Policy Purpose

Merrill Gardens takes appropriate measures to protect both residents and team members from infections and communicable diseases. We stringently follow regulatory guidelines developed by the Center for Disease Control (CDC), the Occupational Health and Safety Administration (OSHA), recommendations from the US Public Health Service, and all appropriate state and federal regulations as well as our own policies and procedures. Please see the Safety Manual for additional information.

Taking these precautions in performing work duties helps to:

- Protect residents, team members, families and visitors from acquiring an infection from someone else (cross infection).
- Protect residents who have had an infection from becoming infected a second time (re-infection).
- Provide residents, other team members and visitors with a safe environment that is as free as possible of infectious materials.

All residents, and team members where required by law, will receive a health screening prior to residing in, or being employed by, the Community. This screening may include a TB test where required by law. Where permitted by law, an LPN/LVN or RN employed by the Community may do the screening.

Definitions

Blood

Human blood, human blood components, and products made from human blood.

Bloodborne Pathogens

Pathogenic microorganisms that are present in human blood and which can cause disease in humans. These pathogens include, but are not limited to, Hepatitis B virus (HBV), Human Immunodeficiency Virus (HIV), and Acquired Human Immunodeficiency Syndrome (AIDS).

Contaminated

The presence, or the reasonably anticipated presence, of blood or other potentially infectious materials on an item or surface.

Contaminated Laundry

Laundry that has been soiled with blood or other potentially infectious materials, or may contain contaminated sharps.

Contaminated Sharps

Any contaminated object that can penetrate the skin including, but not limited to, needles, scalpels, broken glass, broken capillary tubes, and exposed ends of dental wires.

Decontamination

The use of physical or chemical means to remove, inactivate, or destroy bloodborne pathogens on a surface of an item to the point where they are no longer capable of transmitting infectious particles and the surface or item is rendered safe for handling, use, or disposal.

Exposure Incident

A specific eye, mouth or other mucous membrane, or parenteral contact with blood or other potentially infectious materials that may result from the performance of a team member's duties.

Occupational Exposure

Reasonably anticipated skin, eye, mucous membrane, or parenteral contact with blood or other potentially infectious materials that may result from the performance of a team member's duties.

Other Potentially Infectious Materials:

These include:

- The following human body fluids: semen, vaginal secretions, cerebrospinal fluid, saliva, blood, and all body fluids in situations where it is difficult or impossible to differentiate between body fluids;
- Any unfixed tissue or organ (other than intact skin) from a human (living or dead); and
- HIV-containing cell or tissue cultures, organ cultures, and HIV- or HBV- or AIDS-containing culture medium or other solutions; and blood, organs, or other tissues from experimental animals infected with HIV, AIDS or HBV.

Pandemic

A virulent human virus that causes a global outbreak, or pandemic, of serious illness. Because there is little natural immunity, the disease can spread easily from person to person.

Parenteral

Piercing mucous membranes or the skin barrier through such events as needlesticks, human bites, cuts and abrasions.

Personal Protective Equipment

Specialized clothing or equipment worn by a team member for protection against a hazard (i.e., gloves, gown, mask). General work clothes (e.g., uniforms, pants, shirts, or blouses). not intended to function as protection against a hazard, are not considered to be personal protective equipment.

Regulated Waste

Liquid or semi-liquid blood or other potentially infectious material; contaminated items that would release blood or other potentially infectious materials in a liquid or semi-liquid state if compressed; items that are caked with dried blood or other potentially infectious materials and can release these materials during handling; contaminated sharps; and pathological and microbiological wastes containing blood or other potentially infectious materials.

Source of Infection

Any individual, living or dead, whose blood or other potentially infectious materials may be a source of occupational exposure to the team member.

Standard Precautions

An approach to infection control. According to the concept of Standard Precautions, all human blood and certain human body fluids are treated as if known to be infectious for AIDS, HIV, HBV, and other pathogens.

Work Practice Controls

Controls that reduce the likelihood of exposure by altering the way a task is performed.

Guidelines**Protection from Exposure to Pathogens****GENERAL: STANDARD PRECAUTIONS AND BIOHAZARD WARNING LABELS**

Standard precautions are observed to prevent contact with blood or another potentially infectious materials. All body fluids are to be considered potentially infectious.

Containers of regulated waste, or other potentially infectious materials shall be labeled as a biohazard.

HAND WASHING

Team members will wash hands immediately with soap and water, or as soon as feasible after removal of gloves or other personal protective equipment.

Team members will wash hand hands and any other skin with soap and water, or flush mucous membranes (eyes, mouth, nose) with water immediately or as soon as feasible following contact of such body areas with blood or other potentially infectious materials.

Eating, drinking, smoking, applying cosmetics or lip balm, and handling contact lenses are prohibited in work areas where there is a reasonable likelihood of occupational exposure.

TEAM MEMBER SANITATION GUIDES

As part of the initial team member orientation, each team member is instructed in proper sanitation procedures and bloodborne pathogens. Guidelines include:

- Wash hands often during the day with soap and water.
- Wash hands after smoking, before and after using the restroom, handling waste, providing "hands on" care to a resident, upon entering the kitchen, or any time before handling any food, both raw or cooked.
- Wear a hair covering at all times when working around food. A hair net or cap completely covering the hair is acceptable.
- Wear clean gloves and use appropriate serving utensils when handling any food.

PERSONAL PROTECTIVE EQUIPMENT

Appropriate personal protective equipment in the appropriate sizes are readily accessible. Such equipment may include: gloves, gowns, laboratory face shields or eye protection, masks and booties. All personal protective equipment will be considered "appropriate" only if it does not permit blood or other potentially infectious materials to pass through to or reach the team member's skin, eyes, mouth, or

other mucous membranes under normal conditions of use, and for the duration of time which the protective equipment is used. Examples of Personal Protective Equipment and their uses include:

- **Gloves:** Under Standard Precautions, gloves should be used when touching blood, body fluids, secretions, excretions, contaminated items and for touching mucous membranes and non-intact skin.
- **Gowns:** Under Standard Precautions, a gown should be used during procedures and resident care activities when contact of clothing and/or exposed skin with blood, body fluids, secretions, or excretions is anticipated.

When using personal protective equipment:

- All used personal protective equipment is to be removed prior to leaving the work area.
- When personal protective equipment is removed, it is placed in an appropriately designed area or container for storage, washing, decontamination and/or disposal.
- Re-usable equipment should be disinfected immediately. Contaminated materials should be disposed of in appropriate disposal bags or containers for safe removal from the community.

HOUSEKEEPING

Follow these guidelines when completing housekeeping-related tasks:

- All equipment, environmental, and working surfaces are cleaned and decontaminated when surfaces are overtly contaminated or after any spill of blood or potentially infectious materials with a solution of 1:10 bleach or appropriate cleaning product. Contact your chemical supplier for other suitable disinfectants for surfaces or materials that should not be bleached, such as carpeting or furniture.
- All bins, pails, cans and similar receptacles intended for reuse which have a reasonable likelihood for becoming contaminated with blood or other potentially infectious materials should be inspected, cleaned and decontaminated on a regularly scheduled basis, and decontaminated immediately or as soon as feasible upon visible contamination.
- Broken glassware, that may be contaminated, should not be picked up directly with the hands. It should be cleaned up using mechanical means, such as a brush and dustpan, tongs, or forceps.

LAUNDRY

Follow these guidelines for handling soiled or contaminated laundry:

- Contaminated laundry is handled as little as possible.
- Laundry that has been contaminated with any body fluid is placed and transported in labeled bags to protect those coming into contact with potential infectious waste.
- Team members who have exposure to contaminated laundry will wear protective gloves and other appropriate personal protective equipment to prevent contamination of skin surfaces and personal clothing.
- All soiled laundry shall be handled and stored separately from clean laundry and washed immediately.

NEEDLES AND SHARPS

Follow the guidelines below when working with needles and sharps:

- Contaminated needles and other contaminated sharps should not be bent, recapped, or removed from containers. Shearing or breaking of contaminated needles is prohibited. Safety syringes must be used if team members are handling them.
- The Community may provide sharps containers to residents who are self-injecting in their apartments. Sharps containers are also available in medication rooms.
- Contaminated sharps are placed in sharps containers and mailed back or picked up by an appropriate local medical waste disposal source when full.

Team Member Guidelines for Infection Control and Bloodborne Pathogens

Refer to Safety Manual "Bloodborne Pathogens"

Team Member Guidelines for Handling Infectious Waste

Objects that have come in contact with a person who has an infectious or communicable disease can carry or transmit a disease. For that reason, the objects are said to be infectious. Objects identified as infectious waste require special handling, particularly by housekeepers, caregivers and nurses. Such objects include:

- Disposable items used by the resident, and
- Any object contaminated by bodily fluids including saliva, blood, urine, or feces.

Items that should be considered as infectious waste include:

- Soiled bandages
- Used gloves
- Soiled incontinence products
- Used needles and syringes
- Used facial tissue
- Soiled bedding, towels, or clothing, linen

Primary modes of transmission of infectious diseases on the job are:

- Puncture wounds from contaminated needles or other sharp objects;
- Broken skin contact with infected bodily fluids, and/or
- Contact with infectious fluids through mucous membranes of the eyes, nose and mouth.

Reducing the spread of infection is everyone's job. Guidelines include:

- Adhere to Occupational Health and Safety Administration's Standard Precautions.
- Wash hands with soap before and after contact with any resident or contaminated object.
- Follow Community procedures for handling and disposing of infectious waste. Check with a supervisor or General Manager if you are uncertain what to do. (Soiled linen, clothing or towels should be taken to the laundry room in a labeled bag and washed. The bag should then be disposed of or, if reusable, washed appropriately.

- Treat all human blood and certain human body fluids as potentially infectious.
- Assume that all infectious materials containing bodily fluids are contaminated. This includes blood, semen, vaginal secretions, wound drainage, and/or other bodily fluids containing visible blood.
- Handle all used needles and other sharp instruments as contaminated. Place in an approved container designed for sharps. Do not dispose of sharps containers or sharps through normal waste disposal. These items must be disposed of through approved medical waste disposal.
- Always use approved disposal containers for sharps.
- Always wear gloves when touching any bodily fluids, mucous membranes, or any materials or soiled surfaces soiled with bodily fluids. Change gloves after contact with each resident or after handling infectious materials.
- Wash hands immediately following incidental contact with infectious materials and after removing gloves.

Managing the Spread of Communicable Diseases

Preventing further spread of communicable disease is paramount, therefore, follow these steps if a resident or team member becomes ill with a communicable disease:

1. If a team member becomes ill with a communicable disease, the Community will take appropriate action depending on the nature and severity of the illness. This action may include, but is not limited to:
 - Placing the infected team member on sick leave, until the threat of passing the disease to others is over (usually 72 hours after symptoms are resolved).
2. If a resident becomes ill with a communicable disease, the Community will take appropriate action depending on the nature and severity of the illness. This action may include, but is not limited to:
 - Confining the infected resident to his/her apartment and restricting their interactions with other residents.
 - Bringing meals to the infected resident's apartment using disposable food service products and cutlery. All disposable items will be discarded in labeled bags in the resident's apartment before the tray is returned to the kitchen.
 - Requesting that the infected resident temporarily or permanently relocate to a facility that provides a higher level of care, such as a skilled nursing facility or hospital if the Community is not able to meet their needs, or if they place themselves or others at risk.
3. If more than one resident or team member becomes ill at the same time with similar symptoms of a communicable disease, the Community will treat the situation as an Outbreak. If the symptoms coincide with a known pandemic, the same action will be taken. Actions may include, but are not limited to:
 - Placing any infected team member(s) on sick leave, until the threat of passing the disease to others is over (usually 72 hours after symptoms are resolved).

- **Confining any infected resident(s) to their apartment(s), and restricting interactions with other residents. If isolation is required, the resident(s) will be transferred to a suitable setting as soon as possible.**
- **Contacting the Vice President of Operations and informing the local Public Health Department for direction; and state regulatory agencies as required by state regulation.**
- **Investigating the outbreak to identify patterns or other causes. A copy of the community floor plan(s) should be marked with the apartment location of any ill residents. A listing of the date of onset for each person should be maintained with a brief description of symptoms. An attempt should be made to identify any food eaten just prior to the onset of symptoms to rule out food borne illness. Complete any locally required reporting.**
- **Implementing increased cleaning schedules targeting surfaces that are/have been touched by ill team members or residents using appropriate disinfection products.**
- **Restricting visitors to the community as advised by the local Public Health Department or Vice President of Operations. This may be limited to posting a sign at entrances requesting that visitors postpone their visits if they, or a close contact, are ill.**
- **Closing the dining room, until directed by the Vice President of Operations or local Public Health Department, that it is clear to reopen. Residents will be served meals in their apartments using disposable meal service products.**
- **Discontinuing group activities until directed by the Vice President of Operations or the local Public Health Department that they may resume.**

Reporting Communicable Disease

The following is a list of conditions that should be reported to the local or state Department of Health along with any required procedures and forms that must be used. A physician's diagnosis may also be used to assist in identifying an outbreak.

3.5 Exposure to Infectious Material

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Policy Purpose

Merrill Gardens takes immediate actions and supportive measures for anyone exposed to infectious materials. While we employ practices to guard against such exposure, accidents may occur. This policy provides direction on how to handle a situation where exposure to contaminated material is suspected. Exposure procedures are included for residents and for team members (occupational exposure).

Definitions

Contaminated

The presence or the reasonably anticipated presence of blood or other potentially infectious materials on an item or surface.

Contaminated Laundry

Laundry that has been soiled with blood or other potentially infectious materials or may contain contaminated sharps.

Contaminated Sharps

Any contaminated object that can penetrate the skin including, but not limited to, needles, lancets, scalpels, broken glass, broken capillary tubes, and exposed ends of dental wires.

Exposure Incident

A specific eye, mouth, other mucous membrane, non-intact skin, or parenteral contact with blood or other potentially contaminated material that results from the performance of a team member's duties.

Occupational Exposure

Reasonably anticipated skin, eye, mucous membrane, or parenteral contact with blood or other potentially infectious material that may result from the performance of a team member's duties.

Other Potentially Infectious Materials:

- The following human body fluids: semen, vaginal secretions, cerebrospinal fluid, saliva, blood, and all body fluids in situations where it is difficult or impossible to differentiate between body fluids;
- Any unfixed tissue or organ (other than intact skin) from a human (living or dead); and
- HIV-containing cell or tissue cultures, organ cultures, and HIV- or HBV- or AIDS containing culture medium or other solutions; and blood, organs, or other tissues from experimental animals infected with HIV, AIDS or HBV.

Guidelines

What to Do If You Suspect Exposure to Contaminated Material

If it is suspected that exposure to contaminated material has occurred (see definitions above), the General Manager must be notified immediately. The following steps must be taken.

RESIDENT EXPOSURE

The resident should be seen immediately by their primary care provider. If an emergent situation arises, 911 should be called and the resident transported to a hospital.

Document, update, and refer to the following records:

- Resident progress notes
- Medical Health Statement or state physician report form
- Incident Report
- Physician written report or discharge instructions

OCCUPATIONAL EXPOSURE FOR A TEAM MEMBER

See Safety Manual

3.6 Observation and Supervising Residents

.....Revised 10/24/2018 Reviewed 10/24/2018

Policy Purpose

Merrill Gardens strives to promote the safety and health of residents through ongoing observation of health conditions, appearance, personality, and location. This policy describes procedures for documenting changes and taking action when appropriate. It also promotes communication among shifts on a daily basis through documentation, and provides the basis for tracking trends for each resident's condition.

Guidelines

Observation and Supervising – Daily Check In

The resident check-in procedure provides an opportunity for Merrill Gardens' team members to have contact with all residents in the community at least once within each twenty-four-hour period.

AUTOMATED CHECK-IN

Those communities equipped with the Silversphere system have an automated check-in feature accessible to residents from within their apartment. Residents are instructed to utilize the system daily between 5:00 a.m. and 9:00 a.m. to acknowledge that they are checked in. No later than 9:00 a.m. each morning, the system alarms, indicating the end of the check-in period and generates a report that identifies residents who are not checked in. It is printed by a designated team member, who calls each apartment where a resident has not reset their check-in button. After each resident is called, those who have not been reached by telephone are checked on in-person by a designated team member. All phone calls and in-person visits must be documented on the printed report. The completed report is returned to the nurse director or their designee for immediate review. The nurse director or designee will verify that all residents have been accurately accounted for (documented on the report) and signs or initials the report. The report is forwarded to the General Manager (GM), or their designee for review and signature or initials. This report will be placed in a binder located in the GM's office or at the front desk. This procedure will be completed daily.

MANUAL CHECK-IN

Communities that do not have the automated system shall have a manual check-in system in place. A peg system is attached to the upper part of the residents' apartment door. At a designated time in the evening, decided by the General Manager (usually not before 8:00 p.m. or later than 11:00 p.m.) a designated team member walks all floors and sets the pegs in the upright position. When a resident opens their door, the peg falls out of the upright position acknowledging that the resident is checking in. No later than 9:00 a.m. the following morning, a designated team member walks the building with a Daily Resident Check-in Log listing all resident apartments. The team member identifies doors that have

not been opened by the resident as indicated by pegs that are not in the down position. If the peg remains in the upright position, the team member knocks on the door to verify that the resident is alright. Residents who do not answer the door should be called on the telephone, or a designated team member should enter the apartment, to confirm that the resident is alright. All phone calls and in-person visits should be documented on the log. There should be documentation on the log that each resident either had their pin in the down position, was called on the phone, or was physically seen by a team member. If a peg is missing or malfunctioning, the resident must be checked on and a maintenance repair request filled out the same day. The completed log is returned to the nurse director or designee for immediate review. The nurse director or designee will verify that all residents have been accurately accounted for (documented on the log) and sign or initial the log. The log is forwarded to the General Manager, or their designee for review, signature or initial. This log will be placed in a binder located in the GM's office or at the front desk. This procedure will be completed daily.

Once the designated team member has contacted a resident by phone or in-person, they should verify that the peg is in the down position. Any team member who sees a peg up on a resident door after 9:00 a.m. should check on the resident as directed above and notify the front desk of their findings to update the Daily Resident Check-in Log.

IN BOTH SYSTEMS:

Follow these guidelines regardless of which system is in use at the Community:

- The check-in review process begins at 9 a.m. and will take time to complete. The amount of time depends on the system used and the size and configuration of the community. The check-in review and follow-up check process should be completed before noon.
- If a resident has not checked in, review the front desk Resident Sign Out list to confirm whether the resident indicated their intention to leave the Community.
- Residents who prefer to sleep in later than 9:00 a.m. must have indicated this as a part of their service plan and agreed on a time to be checked on. Late checks must also be documented on the Check-in log.
- Call or physically go to the apartment to verify the status of residents who are not checked in. If there is no answer, enter the apartment and check all rooms/areas to confirm that the resident is not there.
- Check resident records to determine whether leaving the premises is part of the resident's identified behavior. Notify the GM or their designee if a resident's location cannot be determined during the initial check-in process. Initiate a search of the building and grounds, including other residents' apartments. See Missing Resident and follow the protocol when a resident leaves the Community and cannot be located.
- Residents whose mental and/or physical condition warrants closer observation should be checked more frequently. The Evaluation and Service Plan should indicate the reasons for, and frequency of, these checks.
- Any change in the appearance, personality or overall health of a resident is noted in the Progress Notes. The resident's responsible party and physician should be notified of any significant changes in the resident's health, appearance or personality, as appropriate.

- After 7:00 p.m. a designated team member walks the halls of the building twice per shift. All exterior exit doors are checked to verify that they are secured. Any event or suspicious activity that is out of the ordinary should be investigated and appropriate action(s) taken. Evening and night team members note their observations and actions in the Communication Log and/or an Incident Report.

Observation and Supervising – E Call System

All communities have an electronic emergency call system.

If the E-call system is not operational, hourly checks on each resident will be completed and documented. The GM or their designee will help ensure that the log(s) of the checks are maintained.

RESPONDING TO E-CALLS

The Merrill Gardens standard is that a team member will respond to an e-call within ten minutes or less. Any time a resident activates the e-call system, a team member must make contact with the resident. The team member shall obtain a verbal response from the resident to determine whether there is an emergency or whether other assistance is requested or needed. If the resident appears to be asleep, the team member must wake them to obtain verbal confirmation that they are not in need of assistance.

CA: *Team members providing night supervision from 10:00 p.m. to 6:00 a.m. must be awake, familiar with Community emergency procedures, trained in first aid, and available as indicated to assist in caring for residents in the event of an emergency.*

- *Communities caring for less than 16 residents - At least one qualified team member must be on call on the premises and capable of responding within ten minutes.*
- *Communities caring for 16 to 100 residents - At least one qualified team member must be on duty on the premises and awake. Another team member must be on call and capable of responding within ten minutes.*
- *Communities caring for 101 to 200 residents - At least one team member will be on the premises, awake; one team member will be on call and capable of responding within ten minutes, and one team member will be on call on the premises.*
- *For every additional 100 residents or fraction thereof, at least one additional team member will be on duty on the premises and awake.*

At least one team member must be on duty, on the premises and awake at all times.

MONITORING E-CALLS

There are several different types of systems with various monitoring capabilities. In some communities, the system is fully monitored by web-based software and accessible from a console and/or from a computer. Other communities have varying degrees of computer monitoring ranging from paging to electronic logging of incoming e-calls. The capabilities of the system will determine how much monitoring intervention by team members is required.

CA: *At least one night shift team member will be located on the premises to enable immediate response to the emergency signal system. If the signal system is visual only, that person must be awake.*

COMPUTER MONITORING LOGS

When e-calls are fully monitored on a web-based system accessible from a console and/or a computer, all events are logged along with response times. The system has several capabilities. A report detailing each call and response times can be printed on a daily basis. Alternatively, the nurse director or their designee can log into the system and review any calls that have a response time greater than ten minutes. For either process, the nurse director will review the log and follow up to determine the circumstances of calls with response times over ten minutes. If additional training or changes are required, they will make a notation on/in the log. The GM or their designee will review and sign off on the log on a daily basis, whether electronic or on paper.

MANUAL LOG

When the e-call system is not capable of producing a printed report of e-calls and response times, a Manual E-Call Log will be maintained. A current Manual E-Call Log should be kept with the AL Communication Book in the Assisted Living work area. Any team member responding to an e-call should make an entry into the manual log as soon as practical after responding to the call, but in all cases before they end their shift. Accuracy in reporting the time of call(s) and response(s) is extremely important. The nurse director or designee will review the log and document the circumstances of calls with response times over ten minutes. Documentation should include the reason the response exceeded ten minutes. If additional training or changes are required, they will make a notation on the log. The GM or their designee will review and sign off on the log on a daily basis.

Photo Identification Album

All current residents will have their photo in the Emergency Information book/file. The resident's first and last name, and apartment number, should be indicated with the photo to aid in identification. The photo will also be uploaded electronically into EHR software for purposes of service planning. The paper or electronic photo may be used by team members and, if necessary, by law enforcement agencies to aid in identifying a missing resident.

SC: *Resident photographs shall be at a minimum two and one-half inches by three and one half inches (2 ½ by 3 ½ inches) in size, dated and no more than twenty-four (24) months old unless significant changes in appearance have occurred necessitating a more recent photograph.*

Retention of Documentation

The Emergency Call logs and the Resident Check-In logs are not part of the resident record. They are for internal quality assurance purposes only. As such they are not retained with the resident records.

Logs for the preceding six months should be kept in a binder in the GM office or at the front desk for internal quality assurance review.

Logs will be retained for a period of seven years, or longer if required by state regulations.



Installation of Resident Check-In Pegs

Materials needed for each apartment door:

1" Screw – 6 x 20 self-drilling hex head

1/8" x 2" Cotter pin

1/8" x 1/2" Plastic tubing or model airplane fuel tubing

Directions for Installation:

1. Put the screw in the eye of the cotter pin and drill into the outside door frame on the doorknob side, about 3 inches from the top of the doorframe (figure 3). Leave enough of the screw protruding to allow the pin to flip freely.
2. Slide the tubing over the unattached end of the cotter pin to protect the door from scratching. (figure 2)
3. Flip the cotter pin up, so the end with the plastic tube leans against the door. When the door opens, the pin will fall to indicate the door has been opened.

Time saver:

Make a jig out of wood measuring $\frac{3}{4}$ " x 4" x 4" (Any kind will do – plywood, fir, etc.). Drill a 7/64" hole (figure 1).

How to use:

Lay the jig flat against the door frame. Slide it up against the top of the jamb and butt it against the door (doorknob side) so the hole is 1 1/2" away from the door and 3" down (figures 3 & 4).

Hold the jig in position with one hand and drill the hole in the door frame. This will eliminate having to measure each door.

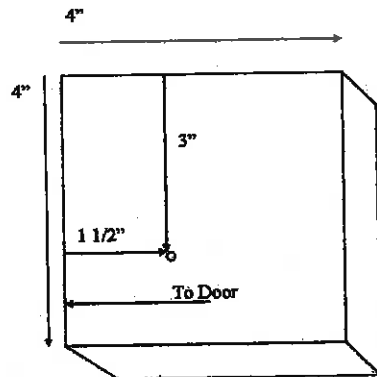


Figure 1

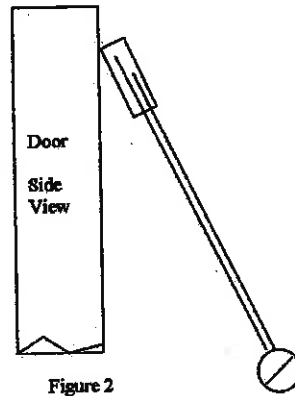


Figure 2

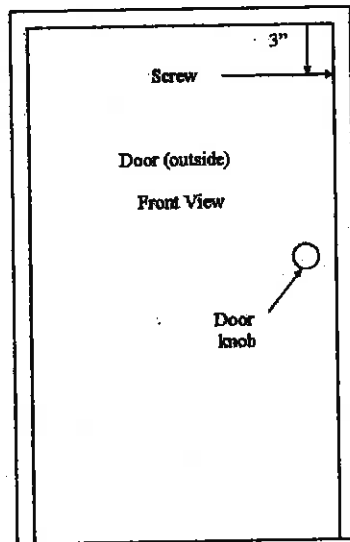


Figure 3

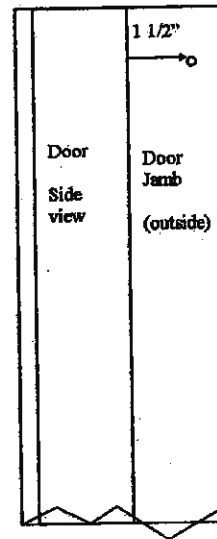


Figure 4

NOT TO SCALE



STATUS LEGEND:

- 1= Breathing and able to Respond Verbally.
2= Breathing and Non-responsive or in Distress
3= Not Breathing

Date (M/D/Y)	Apt. #	Time Called	Time Answered	Reason for call or Service Provided						Resident Status Upon Arrival	Initials
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
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		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
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		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	
		-	-	☐ Fall	☐ Emergency	☐ Personal Care	☐ Other:			-	

Identify Initials with Signature:

3.7 Documentation Revised 09/06/2018 Reviewed 09/06/2018

Policy Purpose

Documentation in the Assisted Living resident chart is known as “documentation by exception”. That means that information is documented is unusual rather than routine. Resident Care staff will use the following forms to relay important resident information and facilitate continuity of care.

Guidelines

24 Hour Communication Report

The care staff use this form to document resident events, falls, move ins, move outs, hospitalizations, ER transfers, skin problems, changes in resident’s health or mental status, and other resident concerns which occur during each shift. Keep 24 Hour Communication Report Worksheets in a binder for no more than 30 days and then destroy them.

Who	What
All care staff	Document resident events, falls, move-ins, move-outs, hospitalizations, ER transfers, skin problems, changes in resident's health or mental status, and other resident concerns which occur during each shift, on this worksheet.
All care staff	Transfer unresolved issues to the next day’s worksheet for continued monitoring.
RCD/Designee	Review and sign this worksheet at the beginning of their respective shifts.
RCD/Designee	Transfers significant concerns to the resident’s Service Notes (see Service Notes section, below).
RCD/Designee	Notifies resident's responsible party and physician if there is an event or change in condition.

Alert Charting

Assisted Living team members will document after a resident has had a change in condition, or unusual or significant events, once a day to follow up on the resident's response.

Who	What
RCD/Designee	Enters the resident info on the Alert Charting Log.
Care Staff	Reviews Alert Charting Log each shift and documents on resident status in the progress notes.
RCD/Designee	Reviews resident progress notes, incident reports, etc. and completes a progress note to conclude alert charting.

Progress Notes

Assisted Living team members will document changes in the resident's condition, or unusual or significant events, and include action taken with resident response. Communication with the responsible person and the physician or medical health professional should also be included. Entries are to be made in black ink, dates with the month, day and year and time of day if possible. Entries must be initialed with the identifying signature on the same page. Maintain Progress Notes as part of the resident record.

SC: In instances that involve significant changes in a resident's medical condition and/or the occurrence of a serious incident, progress notes shall be documented at least daily until the condition is stabilized or the incident is resolved. In all other instances, progress notes for residents shall be documented at least monthly.

Correcting Documentation Errors

Documentation errors are to be corrected by the person making the error. A single line is to be drawn through the incorrect documentation. The date and initials of the person making the correction should be written near the lined-out documentation.

Care Alert Form

Use the Care Alert Form to communicate NON-emergency concerns to the Resident Care Director. Maintain Care Alert forms in the resident care record. Bring urgent or emergency concerns immediately to the attention of the Resident Care Director or designee.

Who	What
All staff	Complete the Care Alert form to communicate NON-emergency concerns to their Resident Care Director.

RCD/Designee	Receives Care Alert form and identifies and completes appropriate follow-up activity. Completes new evaluation within 72 hours if warranted.
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Progress Notes

Resident Name: _____ Apt #: _____

[illegible]

Initials _____ Signature _____

Initials _____ Signature _____

als **Signature**

Initials _____ Signature _____

Initials

Signature

Initials

Signature



3.8 Changes in Condition

..... Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Observing residents on an ongoing basis provides the Community with the ability to respond appropriately to health and cognitive changes. Team members observe residents to identify and respond to changes in health or cognition.

GA: In the case of an accident or sudden adverse change in an assisted living resident's condition or adjustment, the Community must immediately take the actions appropriate to the specific circumstances to address the needs of the assisted living resident, including notifying the representative or legal surrogate, if any. The Community must retain a record of all such adverse changes and the Community's response in the resident's files. The community will keep incident/accident reports on all residents. Whenever a resident is accidentally injured or has an adverse change in physical condition, the representative or legal surrogate must be notified immediately, after obtaining needed care. This report may be in writing. If not in writing, the Community must maintain documentation of the notification. See also GA Investigation and Reporting, GA Major and Minor Medical Emergencies, and/or GA DNR Order.

Definitions

Significant Change

A difference in a resident's ability to perform an activity of daily living based on the Evaluation language that persists longer than two weeks.

Guidelines

Observing a Change in Condition

Team members are responsible for observing, documenting and reporting changes in the physical and cognitive health and wellness of residents. If a significant change is noticed, the team member should complete a Care Alert describing what was observed and report the observation to their supervisor. Significant changes require re-evaluation of the resident and generation of a new or updated Service Plan.

GA: If a resident develops a significant change in physical or mental condition, the Community must obtain medical information necessary to determine that the resident continues to meet the retention requirement(s) and the Community is capable of meeting the resident's needs.

Handling a Change in Condition

There are a number of subtle or obvious conditions or changes in condition that may indicate a serious medical problem (sample list follows). Team members will have ready access to phone numbers for

emergency medical personnel, and resident emergency and medical information contained in the Emergency Contact book/file. If the change is significant and noticeable, notify the:

- Community nurse, if available, to evaluate the situation
- Attending physician or, if an emergency, call 911
- Community General Manager (GM) or their designee
- Resident's responsible party. Depending on the seriousness of the change or issue, the GM or their designee may want to notify the family personally.

Details of the notification, including the date, time and relationship of the individuals notified, should be documented in the resident's record.

When in doubt, team members should err on the side of caution. Team members should assume that an observed change is important. Do not make a diagnosis.

SC: The administrator or his or her designee shall report changes in a resident's condition, to the extent that serious health concerns and/or injuries, e.g., fracture, behavioral changes or heart attack, are evident, to the attending physician and the responsible party immediately, not to exceed twenty-four (24) hours, consistent with the severity or urgency of the condition in accordance with community policies and procedures.

Notes of observation shall be documented at least daily until the condition is stabilized or resolved. In all other instances, notes of observation for residents shall be documented at least monthly.

Transferring a Resident: Notifications

Notification of the resident's responsible party should be made prior to non-emergent transfers. Notification may be written or verbal. A written record of verbal notifications must be documented in the resident record.

In emergency situations, especially those involving a 911 call, a resident may be transferred from the Community without prior notification of the responsible party, which is usually done by the Community General Manager or a designated team member. If this occurs, the GM or designated team member should notify the responsible party as soon as possible after the transfer.

Condition Changes

Team members quickly gain knowledge about individual residents in the Community. Detailed documentation of changes in condition helps to maintain a high level of awareness by the entire team and helps to protect the resident. Team members should observe the resident for a decline in condition or ability. These may include:

BEHAVIOR

- Wanting to sit up to sleep;
- Confusion;
- Depression; (symptoms may include decreased appetite, withdrawal, decline in personal appearance, etc.)
- Agitation or aggressive behavior;
- Requesting room service instead of coming to the dining room for more than three (3) consecutive days;

- Poor personal hygiene (body odor, soiled clothing, evidence of poor grooming, etc.) If a resident exhibits poor personal hygiene, the GM must intervene to evaluate the situation and to help address the resident's change in habits.

COGNITION

- Personality changes;
- Visual or auditory hallucinations;
- Confusion;
- Repetitive behavior, questions;
- Disorientation to person, place or time.

HEALTH

- Shortness of breath or a persistent cough;
- Complaints of indigestion, pain in the jaw, chest or arm, or tightness in the chest;
- Reduced appetite or skipping meals;
- Bladder or bowel incontinence;
- Constipation;
- Loose stools or diarrhea;
- Nausea or vomiting;
- Dehydration (may be indicated by confusion, dry mouth, etc.);
- Headaches;
- Insomnia or night wandering.

PHYSICAL

- Pale skin or blue around the lips;
- Rashes;
- Unsteady walk, or falls;
- Drooling;
- Lip smacking;
- Shaking;
- Slurring of speech, or word finding difficulty (aphasia);
- Complaints of pain or tenderness.



Care Alert

The Care Alert form may be used by any Team Member to report resident behavior or issues they observe or are told about. The completed form is to be given to the Team Member's supervisor. The supervisor is to bring the form to the daily management stand-up meeting for discussion. The General Manager will determine which manager is to follow up on the issue. Completed Care Alert forms for residents living in licensed apartments will be retained in their chart. Completed forms for residents living in unlicensed apartments will be kept in the resident file in the business office.

Note: The Care Alert form is not intended to be used to report urgent or emergent resident issues. Those issues must be reported immediately to the Team Member's supervisor. If a resident appears to be having a medical emergency, 911 is to be called.

Resident Name:

Apt. Number:

CHECK ALL CONCERNS THAT APPLY:

☐ Behavior (negative or change)

☐ Odor or grooming issue

☐ Interaction with other residents

☐ Inappropriate or soiled clothing

☐ Making negative statements about self or others

☐ Medications not stored in secure area

☐ Not participating in activities

☐ Change in physical ability

☐ Missed Meals

☐ Change in speech

☐ Staying in apartment

☐ Change in memory or confusion

☐ Apartment dirty

☐ Other (describe)

DESCRIPTION OR COMMENTS (OPTIONAL):

Reported to Team Member By: ☐ Team Member ☐ Resident ☐ Outside Provider ☐ Family ☐ Visitor
Team members please turn this completed form into your supervisor today.

Signature of Person Completing Form:

Printed Name:

Date:

Date Reviewed:

Manager Assigned:

Action Taken:



Physician Notification of Resident Condition

Community: _____

Address: _____

Phone Number: _____

Return Fax Number: _____

HEALTH CARE PROVIDER: Click or tap here to enter text.

FAX: Click or tap here to enter text.

Date: Click or tap here to enter text.

Resident: Click or tap here to enter text.

This is a formal notification that the resident listed above has

- ☐ Had at least one fall in this community **OR**
- ☐ Attempted to elope from the community **OR**
- ☐ Has an area of skin breakdown on: Click or tap here to enter text.

The resident has been evaluated by the community for

- ☐ risk of falling ☐ risk for elopement ☐ risk of skin breakdown

and interventions have been implemented to attempt to reduce the risk. The interventions that have been put into place and care planned so far for this resident include:

- Click or tap here to enter text.
- Click or tap here to enter text.
- Click or tap here to enter text.

Due to the fact that this resident has had the above named incident, it is the community's position to ensure that the physician is aware of his/her situation, and what measures the community is taking to prevent future incidents.

Community Representative: _____ Title: _____

Physician Acknowledgement

As this resident's physician, I acknowledge that the Community has identified this resident as

- ☐ At risk of falling ☐ At risk for elopement ☐ At risk for skin breakdown

- ☐ I am recommending the following as new orders for this resident to decrease the risk of future incidents:

OR

- ☐ I feel that there is no need for change in the resident's medical plan of care at this time.

Physician: _____ Date: _____

Form to be faxed to physician and returned to community. If no response is obtained, a call should be placed to the physician.

3.9 Weight Management.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

To establish a system for monitoring resident's weight status and intervening when a resident receiving assisted living services, Garden House services or residing in a licensed apartment, is at nutritional risk in accordance with state and local regulations.

Definitions

A resident may be at nutritional risk based on the following criteria:

- Recent unintentional weight loss or gain of 10 % in six months, 7.5% in three months, or 5% in one month.
- Absence from eating in the dining room for over a week.
- Resident, team member or responsible party report an ongoing lack of appetite, chewing or swallowing issues, and/or weight loss or weight gain.

Guidelines

Obtaining Weights

1. Residents will be weighed as follows:
 - All residents will be weighed at move-in. Thereafter, assisted living residents will be weighed at the time of their routine re-evaluation.
 - Residents at nutritional risk will be weighed by the care team on a monthly basis. Residents will be weighed more frequently if the severity of nutritional risk warrants such intervention.
 - Residents of the Garden House will be weighed monthly.
 - Any exceptions to this schedule will be documented in the Service Plan.
2. All documentation related to weight management must be completed by the care team. It is the responsibility of all dining services team members to advise the Executive Chef (EC) of any observed or reported concerns, who will in turn notify the Community nurse director.
3. Weights will be recorded in the electronic health record (EHR).
4. Upon discharge, the resident's EHR weight log should be printed and filed in the resident's record.

Unintended Weight Loss or Gain

1. Residents at nutritional risk will have:

- Documented notification to the resident's health care provider (physician), and responsible party.
 - A nutritional intervention requested from the health care provider.
 - Caregivers and Dining Service team members advised to report any changes in resident's eating habits to their supervisor.
 - At least a monthly weight.
 - Service plan(s) revised to reflect the resident's current status and associated interventions.
2. The Community nurse director is responsible for monitoring residents at nutritional risk. Additionally:
- Team members will document the resident's current weight status, eating habits, skin condition/physical issues and/or any other pertinent nutritional interventions in a monthly progress note.
 - The resident's primary care provider will be notified of any significant issues or changes.



3.10 Oxygen.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens encourages residents to live as independently as possible. To facilitate this independence, we allow residents to use and store oxygen in the Community per the Compressed Gas Association (CGA) guidelines, and state and local regulations. Residents using oxygen in the Community must be able to self-manage the administration of their oxygen. This may be accomplished through the use of an outside agency or care provider. The Community has the discretion to allow the use and storage of oxygen based upon the resident's ability to self-manage administration and the Community needs.

Definitions

Gaseous Oxygen

Oxygen a gaseous state that is a colorless, odorless, tasteless gas. It is heavier than air and supports combustion.

Liquid Oxygen

Oxygen that is in a liquid state that is light blue in color and flows like water. It is heavier than water and supports combustion. Has cryogenic high pressure producing expansion properties.

Oxygen Concentrator

Extracts oxygen molecules from the room air.

Resident

Includes resident-arranged care and/or equipment providers for the purpose of self-management.

Transfiling

Moving oxygen from one container to another. Usually refers to filling portable containers from a base unit.

Guidelines

General Safety Rules

General safety rules for all types of oxygen use include:

- "No Smoking" signs posted outside the resident's apartment and any oxygen storage area(s). Smoking is not allowed in any portion of the Community.
- An oxygen tank must have a secured base when in use or stored. Oxygen canisters are under high pressure (up to 2000 psi) and may burst if damaged.

- Avoid using oil, including petroleum jelly (Vaseline), grease or hand lotions containing petroleum-based ingredients and other materials that easily ignite around oxygen.
- Avoid using absorbent materials, such as clothing or bedding, that become saturated with oxygen and easily ignite.
- Avoid using wool and synthetic materials close to the oxygen supply.
- Oxygen equipment should be kept at least five feet away from open flame, sparks, or any heat source such as stoves or heaters.
- Avoid using electrical equipment close to the oxygen supply (within five feet).
- Electrical equipment should be turned off prior to unplugging the equipment.
- Liquid oxygen is very cold (-181.4 F) and can cause freezing or burning of skin. Avoid contact with skin or clothing.
- If liquid or gaseous oxygen is leaking, evacuate the room immediately, open windows, and call the oxygen supplier.
- If a fire occurs in an area containing oxygen, evacuate the area, call 911 and advise them that oxygen is on the premises.

GA: *All staff will be trained in the safe handling of oxygen, in safety practices to be observed during oxygen administration, and in the Community's protocol for oxygen administration.*

Physician's Orders

A physician's order is required for oxygen use by a resident. The resident's ability to self-manage will be evaluated. New orders will be obtained annually or as required by state regulations. A copy of the order must be kept in the resident's chart. The order must include:

- The rate of flow
- The means of administration
- Humidification
- Under what circumstances oxygen is used
- A statement that the resident is able to self-manage oxygen

Only the resident or a licensed nurse may change the rate of oxygen flow in accordance with physician's orders. The physician must be notified if the resident has a change in condition or exhibits behavior that may indicate difficulty with self-management.

Oxygen Storage

Residents may store oxygen in their apartments in compliance with CGA guidelines and state and local regulations:

- Do not subject containers to extremes of heat and cold.
- Protect containers from a corrosive atmosphere.
- Store containers in an upright position.
- Containers must be secured to prevent tipping.

- Store in a well-ventilated area away from combustible materials.

Liquid Oxygen

Residents living in a Merrill Gardens community are prohibited from using liquid oxygen. This does not apply to residents' visitors with a portable tank. Team members may assist potential residents with finding alternatives to liquid oxygen.

Shared Apartments

In apartments where multiple residents reside, all occupants of the shared apartment must be able to understand oxygen precautions and safety procedures.

3.11 Care of Residents with Cognitive Impairments

..... Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens generally accepts residents experiencing cognitive impairment, such as early onset and mild to moderate stage symptoms of Alzheimer's disease and related dementias, if behavior:

- Poses no threat to the resident's health and safety
- Poses no threat to the health and safety of other residents and/or team members
- Does not disrupt the operation of the Community

Specially designed and licensed Merrill Gardens Communities may handle Alzheimer's disease and related dementias as a primary mode of operation.

Guidelines

Evaluation and Training

Follow these guidelines related to the evaluation of residents for cognitive impairment:

- Cognitive impairment is evaluated through the EHR software. The care and supervision provided to the resident is personalized to meet the resident's needs. The Evaluation will be used to identify residents who may be at risk for elopement.
- Residents who exhibit tendencies to wander away, and/or who are unable to return to the Community unassisted, will not be retained in an unsecured setting.
- The Community team periodically receives training in recognizing signs of cognitive impairment, including Alzheimer's disease and related dementias and/or how to handle the behaviors associated with the disease.

Identification and Monitoring

Follow these guidelines for the initial and ongoing identification and monitoring of residents:

- All current residents will have their photo in an Emergency Information book/file at the front desk. The residents first and last name and apartment number should be indicated with the photo to aid in identification. The photo will also be uploaded electronically into company software for purposes of service planning. The paper or electronic photo may be used by staff, and if necessary, by law enforcement agencies to aid in identifying a missing resident. Residents who have a history of wandering will have this information included on their Service Plan.
- Residents are required to sign in and out of the Community as they come and go.

- If a resident who is unable to leave the Community without supervision attempts to do so, team members should attempt to stop and redirect the resident to an activity inside the Community. If the resident continues to attempt to leave, a team member should go with them.
- If a wandering incident occurs, the resident will be re-evaluated for appropriateness and retention.
- The primary physician will be notified in writing, using the Physician Notification of Resident Condition form, about any residents identified as at risk or who has attempted to elope. Any written response from the physician will be implemented and filed in the resident record. A phone call should be made to the physician (and documented) if a response has not been received.

3.12 Intimacy and Sexuality.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Humans have a need for intimacy. Not all forms of intimacy are sexual in nature. Intimacy and sexuality are very personal in nature. Personal values, beliefs and attitudes affect how the residents and team members respond to these situations. Team members need to understand the residents' needs as they relate to intimacy and sexuality, while promoting a safe environment for all.

Definitions

Intimacy

The giving and receiving of affection.

Sexuality

Behaviors and tendencies pertaining to sex.

Guidelines

General Approaches

It may be difficult for the resident and/or spouse to initiate a conversation with team members about these issues due to generational and cultural differences. There are basic concepts team members should be aware of to facilitate appropriate communication and behavior that include:

- Approach the topic with the resident and/ or their responsible party, rather than waiting for them to initiate communication. Team members should offer information that may assist in communicating about these issues.
- Give permission to talk about sex.
- Encourage approaches that foster non-sexual intimacy.
- Adjust routine and bedtime to coincide with the resident's need for intimacy.
- Recommend professional help if appropriate.
- Avoid the use of medications to control behavior.

Effects of Dementia on Intimacy

For the Resident:

- Loss of desire
- Increased interest
- Misidentification of a spouse as someone else

- Misidentification of others as a spouse
- Mistaking personal care as sexual in nature

For the Spouse or Significant Other:

- Exhaustion
- Grief
- Jealousy
- Loss of attraction
- Sense of duty

Non-Sexual Behaviors That May Appear Sexual

Loss of social inhibitions and insight about behavior. Examples include:

- Urinating or defecating in public locations.
- Removing clothing in public locations.

Fondling of genitals:

- May indicate the need to use a bathroom.
- May indicate urinary tract (UTI) or vaginal infection.
- May indicate skin irritation or rash.
- May indicate uncomfortable clothing.

Non-sexual intimacy. Some examples are:

- Hugging.
- Holding hands.
- Dancing.
- Massage.

Approaches When Resident Alone Is Involved

Residents may masturbate to meet their sexual needs. It is the team member(s) responsibility to protect the residents' privacy and dignity. The below steps may be taken, as appropriate:

- Provide for and allow privacy.
- Do not shame or tease the resident. This is not an abnormal behavior.
- Observe the resident for UTIs and skin rashes, which may be the cause of the behavior, rather than a need for sexual stimulation.
- If the behavior is occurring in inappropriate places:
 - Increase non-sexual physical contact.
 - Use deterrents if needed, such as athletic cup, fanny packs, or clothing that is more difficult to remove.

Approaches When Two Residents Are Involved

Residents have a right to intimacy. Residents with dementia may not be capable of informed consent. The responsibility of the team members is to report if both residents do not appear to be involved willingly. A secondary responsibility is to assist with privacy and dignity. If two residents are involved in an intimate relationship, follow these steps:

- Observe the response of both residents to determine if one is not comfortable with the situation. If so, separate the residents and follow-up on the event as an incident.
- If both appear to be comfortable with the situation, determine whether privacy is adequate.
- Involve both responsible parties and provide information. Use caution when giving information about other residents, so that confidentiality is not compromised.
- Notify physician(s) of the situation and document any response.
- Update the resident(s) Service Plan(s) to reflect acknowledgement of the situation.
- Avoid the use of medications to control behavior.

3.13 Continence and ToiletingRevised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Independent toileting habits should be promoted. In some cases continence can be maintained with toileting assistance from staff. Residents with dementia may be incontinent for the same reasons as other residents, but there are also reasons that are specific to the disease process.

Definitions

Continence

The ability to retain urine and/or feces until a proper time for their discharge.

Guidelines

Common Causes of Incontinence

There are numerous possible causes for incontinence. If the cause can be identified, it can sometimes be eliminated or managed. Below are some of the common causes of incontinence:

- **Medical Conditions**

Physical problems such as a urinary tract infection (UTI), vaginal infection, enlarged prostate, or fecal impaction that may result from illness or disease and may contribute to incontinence.

- **Stress Incontinence**

A decrease in muscle tone controlling the bladder that may cause leakage when laughing, coughing, sneezing or standing up.

- **Bladder Irritants**

Certain beverages can cause irritation to the bladder, creating a sensation of fullness and urgency to urinate. They may also cause painful urination. Common irritants are coffee, alcohol and citrus juices.

- **Inadequate Hydration**

Residents may not remember to drink fluids, causing the urine to become more concentrated. This may irritate the bladder, leading to incontinence and potentially to infection.

- **Medications**

- Diuretics cause increased urine production that can make it difficult to get to the bathroom frequently enough to avoid incontinence.

- Some medications contain stimulants like caffeine, which can also irritate the bladder.
- Anti-hypertensive medications may also irritate the bladder.
- Sedatives, tranquilizers and hypnotics can reduce the resident's awareness of the need to urinate or defecate.
- Anti-Parkinsonian medications and antihistamines may cause urinary retention, which can lead to UTIs or dribbling.
- **Functional Incontinence**
Residents with dementia may not be able to interpret the signals their body gives them to tell them they need to urinate or defecate. They also may not remember where the toilet is, or the sequential steps involved in toileting, such as undressing, sitting, waiting, voiding, hygiene, and redressing.
- **Neurological Changes**
Brain damage from disease processes can interfere with the processing of the message from the bladder to the brain telling the resident that they need to use the bathroom.

Fluids

Residents tend to forget to drink fluids. Inadequate fluid intake can contribute to urinary and bowel problems, and incontinence. Fluids should be offered, at a minimum, with every meal and snack. During warm weather, additional fluids should be offered. Popsicles, gelatin and broth are other sources of fluids. Fluids should not be restricted, even in the evening in an attempt to avoid incontinent accidents.

Environmental Considerations

The following environmental cues can be helpful:

- Public bathrooms should be clearly marked in familiar words and symbols. Pictures can be helpful in identifying bathroom locations.
- Bathroom doors in apartments should be left open, so the resident can see the toilet.
- A light may be left on at night to enable the resident to locate the toilet.
- If a commode is used, the lid should be left up, so it looks like a toilet.
- Some residents may have visual difficulty seeing a white toilet seat if the flooring and walls in the bathroom are light colored. A different color seat may make the toilet easier to find.

Promoting Independence

Following are ways to help the resident remain as independent as possible:

- Simplify clothing to make sure it is easy for the resident to get off and on.
- Do not rush the resident. Allow time for them to urinate or defecate.
- As confusion increases, use clear single step instructions:
 - Pull down your pants.
 - Sit down.

- You can go now.
- Wipe yourself.
- Stand up.
- Pull up your pants.
- Wash your hands.
- Identify the cues the resident uses to indicate the need to urinate or defecate.
- Some residents need to have an object to hold to keep them busy while sitting on the toilet.
- Some residents may need to get up and down a few times before they can sit on the toilet.
- Accidents may happen. Use a positive approach to minimize embarrassment and reassure the resident that they will get help with hygiene and clothing.

Toileting

Some residents may need more assistance from team members to manage toileting. For those residents, the following guidelines are applicable:

- **Schedules**

Routine is important in maintaining continence. Develop a schedule for toileting that anticipates and allows the resident to void/defecate before an incontinence episode may occur (reminding or assistance). Before meals and at bedtime are important times of the day. If possible, avoid times when the resident may feel rushed. Toileting every two hours creates undue stress on the resident and team members, and is usually not effective.

- **Cues**

Many residents who are unable to verbalize a need for toileting give physical cues. Team members need to observe and learn the cues the resident gives that can include pacing, rocking, or removing clothing.

- **Night time**

Residents may be particularly confused or disoriented at night. If the resident is able to sleep through the night and remains continent, they should not be awakened for toileting. If awakening a resident for toileting causes agitation or an inability to return to sleep, a protective garment may be indicated at night. Team members who awaken a resident at night should identify themselves and gently state why they are there. The resident may need extra time to wake up and gain their bearings before a team member is able to assist with toileting, or they may become agitated. Adequate lighting is important for toileting at night.

Incontinence

Some residents will not be able to remain continent with or without assistance with toileting. Any time a resident who has been continent becomes incontinent, it signals the need for further evaluation to determine whether it is the result of an underlying medical issue. The resident's primary physician should be notified at the onset of incontinence to rule out underlying medical conditions. If the resident

who is incontinent cannot be managed with reasonable assistance from team members to remain clean and socially appropriate, should be evaluated for appropriateness to remain living in the Community.

There are several approaches to managing incontinence in addition to toileting assistance:

- **Use of Protective garments**

There are many styles and sizes of adult protective garments available. The volume of urine experienced in an incontinent episode and resident comfort will determine the product works best. The use of protective garments is not a replacement for scheduled toileting.

- **Catheter Use**

There is risk of injury from pulling out a catheter. Residents with catheters should be evaluated for appropriateness. Team members are not permitted to insert or remove indwelling catheters. Refer to state regulations for permissible catheter care.

Team members should follow standard precautions for infection control when handling a resident who is experiencing urinary incontinence.

Bowel Incontinence

Bowel incontinence may occur with or without bladder incontinence. The resident's primary physician should be notified at the onset of incontinence to rule out underlying medical conditions. If the resident who is incontinent cannot be managed with reasonable assistance from team members to remain clean and socially appropriate, should be evaluated for appropriateness to remain living in the Community.

Common causes of bowel incontinence include infection, diarrhea, constipation, and/or fecal impaction. Team members should follow standard precautions for infection control when handling a resident who is experiencing bowel incontinence.

Bowel continence can be promoted by ensuring adequate fluid intake, fiber in the diet and exercise. Residents should be encouraged to sit on the toilet long enough to empty their bowels. The use of medications to promote elimination should be avoided.

3.14 Activities of Daily LivingRevised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Residents are encouraged to be as independent as possible in performing their activities of daily living (ADLs). Residents can be overwhelmed with the number of steps required to complete these tasks, are unable to remember why it is important, or may be confused by sequence of the process. Some residents can become fearful and may resist any attempt at assistance from team members. Other residents have physical or mobility challenges that make it difficult for them to be independent with one or more of their ADLs. Grooming and dressing tasks may need to be repeated throughout the day, or may take the entire day to be completed. The responsibility of and challenge for team members is to encourage independence, reduce fear and provide for dignity. This requires flexibility, and in some cases, creativity.

Definitions

Activities of Daily Living (ADLs)

The tasks necessary to bathe, groom and dress.

Guidelines

Encouraging Resident Participation

Following are ways to encourage residents to participate in their ADLs.:

- **Focus on ability**
Create a situation where the resident can be successful. The goal is completing the task. If the resident cannot complete the entire task, assist them with some of the steps, but encourage them to complete the last step to foster a feeling of success.
- **Establish a routine**
Residents need consistency that minimizes the new information they must deal with. Minimizing the number of different team members assisting with ADLs helps to increase trust with residents.
- **Minimize distractions**
Residents may have a difficult time focusing and staying on task. Minimizing distractions increases success in performing ADLs.
- **Encourage independence**
Allow enough time for the resident to complete as much of the task as they can. Provide enough assistance, so they can participate as fully as they are able.

- **Communication**

Speak to the resident as an adult, but keep instructions simple. Use a gentle voice and give one step at a time. Do not argue. If the resident resists, try a different approach, try later or try a different team member.

Assisting with Dressing and Grooming

The manner in which team members assist a resident with dressing and grooming may depend on their stage of cognitive impairment:

- **Residents with physical limitations:**
 - Allow enough time for the resident to complete the task.
 - Check for any adaptive equipment the resident may use.
 - Set up clothing as needed to allow the resident to complete the task as able.
 - Provide assistance as needed or requested.
- **Early Stages of cognitive impairment:**
 - Establish a routine.
 - Keep clothing organized and minimize clutter in the closet and dresser.
 - Discreetly remind the resident to change their clothes or attend to grooming tasks. Be specific.
- **Middle Stages of cognitive impairment:**
 - Allow enough time for the resident to complete the task.
 - Pre-select and lay out clothing for the resident to put on for the day.
 - If picking out clothing with the resident, limit to two choices to minimize confusion.
 - Limit distractions.
 - Provide simple one-step directions. Do not give another direction until the resident has completed the previous one. Face the resident, make eye contact, and speak in a clear, calm voice.
- **Late Stages of cognitive impairment:**
 - Demonstrate for the resident what you want them to do.
 - Give the resident something comforting to hold while completing tasks they are no longer able to accomplish.

Assisting with Bathing

The team member should adjust their approach to assisting a resident with bathing depending on their stage of cognitive impairment:

- **Residents with physical limitations:**
 - Allow enough time for the resident to complete the task.
 - Check for any adaptive equipment the resident may use.

- Set up as needed to allow the resident to complete the task as able.
- Provide assistance as needed or requested.
- **Early Stages of cognitive impairment:**
 - Establish a consistent routine.
 - Provide adequate lighting and warmth in the room.
 - Provide as much privacy as possible.
- **Middle Stages of cognitive impairment:**
 - Prepare the bathing area ahead of time with needed supplies.
 - Keep the bathroom as homelike as possible.
 - Minimize noise and distractions.
 - Outcomes may be more successful if bathing is separated from washing hair.
- **Late Stages of cognitive impairment:**
 - Minimize the frequency of complete baths or showers. Some residents tolerate sponge baths more easily. Changes in the brain can cause pain when water from a shower impacts a resident's skin.
 - Place a washcloth or handheld shower in the resident's hand help them to guide the movements.
 - Gently coach the resident for each step, giving one instruction at a time.

3.15 Resident Support Systems and Dementia

.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

During the residency, families and significant others may be faced with many concerns and questions. They will look to the team members for answers or validation of the decisions they make. The role of team members is to provide information resources and support to the families/significant others.

Definitions

Family

Includes relatives and significant others the resident has identified.

Validation

Acknowledging the feelings or emotions expressed without agreeing, disagreeing or making a judgment.

Guidelines

Helping Families with the Progression of Dementia

Here are some of the ways families will need care and support:

- Families may need help understanding the personality changes that occur with dementia. The resident may become "different" or "difficult" as the disease progresses due to deterioration of the brain. This may result in increased frustration over loss of function, the inability to express needs or feelings, and an increased dependence on others.
- Families may need help understanding that each resident is affected by the progression of the disease process differently and that different approaches may work for different residents.
- Families may need educational resources about the usefulness of multiple approaches to care. Different approaches work at different times, and flexibility is important.
- Families may need help understanding the importance of realistic expectations about the resident. Resident's often appear more capable than they are.
- Families may need support to accept a change in their role. As the disease progresses, residents may need assistance with decision-making or may confuse relationships.

Helping Families Adjust to the Community

Moving a loved one into a Community can be a difficult decision for family members. Here are some of the ways they need support while adjusting to this transition:

- Families need support in their decision to allow others to assist in the care of their loved one, which can be a difficult transition. Team members need to show concern,

professionalism and a desire to work with the family, who may be experiencing feelings of guilt, loss and grief.

- Families need support when the resident's care needs change. Open communication is important to determine the best approach.
- Families may need help understanding that the resident may be resistive to a new service or living environment and that it is normal. Change can be confusing or frightening for the resident.
- Families may need assistance in long-range planning related to care, legal and financial decisions. Team members should offer a variety of resources such as the local Alzheimer's Association Chapter, elder law specialists, or support groups.

Involving Families

Keeping families involved and informed is one of the best ways to help families cope. Here are some ways to do that:

- Invite and encourage families to play an active role in the resident's care at their own pace.
- Keep lines of communication open. Keep the family informed through conversations, care conferences, emails that include pictures of resident participation in activities, calendars of events, and newsletters.
- Make suggestions about how to make visits more rewarding; suggest an activity in which they can participate with the resident.
- Value the information and opinions of the family; the resident may be different, but often the family has insight into the resident's behavior that supports care and enhances successful approaches from the team.
- Reassure the family that they will be notified when changes occur.
- Treat the resident in the manner the family would like them to be treated.
- Make the family feel welcome.
- Assist family members in coping with feelings of loss and grief by listening and validating their feelings.
- Anticipate family needs and concerns-follow up on concerns.
- Notify families about good news, resident participation in activities and events, and care successes.
- Model good communication and interaction skills with the residents and families.
- Be honest with families.
- Maintain a sense of humor, while ensuring the dignity and privacy of the residents.
- Let families know how important and appreciated they are.

3.16 Use of Secured Outdoor Spaces

.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

The use of outdoor spaces by residents of the Garden House is encouraged. Outdoor spaces can help residents with cognitive impairments maintain a connection with the natural environment, while still addressing safety needs. Access to a secure outdoor space allows each resident the opportunity to have choice in their environment. Activities utilizing the outdoor space can provide opportunities for socialization and participation in tasks that are familiar.

Guidelines

Physical Environment

The following elements are required for a secure outdoor area:

- Direct access from the Garden House unit
- Protective perimeter fencing/decking
- Locks on all access gates
- Walking path or area (preferably non-glare, non-slip surface)
- Seating
- Non-toxic plants

The following elements can enhance the use of the secure outdoor area:

- Resident accessible planting areas/boxes
- Shade (umbrella or arbor)
- Table(s)
- Clothesline (recommend retractable or umbrella style)
- Bird bath, mailbox
- Activity spaces (putting green, horseshoe pit, etc.)

Resident Access

Doors from the unit that exit directly to the secure outdoor space will be unlocked during the day. Residents who choose to go outside are encouraged to dress appropriately for the weather. In some cases of extreme weather conditions, it may be necessary to limit access to the outdoor areas of the Garden House to prevent harm to the residents.

Supervision

The integrity of door and gate locks must be checked on each shift when residents have access to the outdoor space. Residents do not have to have a team member accompany them outside in the outdoor secured area(s) of the Garden House. Team members are to make frequent checks on residents who go outside to observe that they are not in distress and to assist those who may need cueing to re-enter the Garden House. The frequency of the checks will be determined by the conditions outside and the abilities of the resident.

Activities

Many activities can be initiated utilizing the outdoor spaces of the Garden House. Adequate seating for the activity or rest periods should be available. Outdoor activities may need to be adapted to be held in the space available. Walking surfaces should be even and sharp changes in elevation from one surface to another should be minimized.